INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Weekly paclitaxel + trastuzumab → trastuzumab continuation

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
Intervals differ by treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00815
Version 3

Paclitaxel weekly for 12 doses with trastuzumab every 3 weeks; continue trastuzumab to complete one year of HER2 treatment when appropriate. The team selects intravenous (IV) trastuzumab or locally approved subcutaneous trastuzumab; these are alternative formulations, not additional medicines.

Treatment stages

01 **Paclitaxel + trastuzumab**
First 12 weeks

02 **Trastuzumab alone**
Complete one year in total

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology promptly for fever, shivering, unusual bleeding, severe mouth or bowel symptoms, inability to drink, reduced urine, new breathlessness or a painful/leaking line. Severe chest pain, collapse or severe breathing difficulty needs emergency help.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your medicines and their different roles

Adjuvant treatment aims to reduce recurrence; confirm the exact treatment setting and sequence with your team.

TAXANE CHEMOTHERAPY

Paclitaxel

Taxol (conventional paclitaxel)



Locks the cell division machinery

Cells use tiny structures called microtubules to separate their chromosomes when dividing. Paclitaxel stabilises these structures so they cannot rearrange normally, interrupting cell division. It can also affect nerves and other healthy tissues.

Microtubules form → Structures remain locked → Division is interrupted

What to watch for

Report tingling or numbness before the next weekly dose, especially if it affects walking or hand use. Tell the nurse immediately about flushing, chest or throat tightness, itching, dizziness or breathing changes during infusion.

HER2-TARGETED ANTIBODY

Trastuzumab

intravenous (IV) infusion or locally approved subcutaneous injection every 3 weeks



Targets HER2 growth signals

Trastuzumab binds to the HER2 protein on cells and helps reduce HER2-driven growth and recruit immune activity. HER2 testing identifies the cancers for which this approach is used.

HER2 on cell surface → Antibody binds → Growth signalling reduced

What to watch for

Keep echo appointments throughout HER2 treatment. Report breathlessness, swelling, palpitations or a reaction; chemotherapy side effects are tracked separately.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

This adjuvant option uses weekly paclitaxel with three-weekly trastuzumab for selected HER2-positive, node-negative cancers no larger than 3 cm. It avoids an anthracycline, but still requires heart monitoring and review for chemotherapy effects.

Treatment setting

Adjuvant HER2-positive, node-negative breast cancer; tumour ≤3 cm

Your exact schedule

Paclitaxel weekly for 12 doses with trastuzumab every 3 weeks; continue trastuzumab to complete one year of HER2 treatment when appropriate. The team selects IV trastuzumab or locally approved subcutaneous trastuzumab; these are alternative formulations, not additional medicines.

Alternatives

Discuss suitable endocrine, targeted or chemotherapy options, expected benefit, practical demands and the option of supportive care according to your setting.

Why this treatment may be chosen

Show trial results and what the numbers mean

- Pembrolizumab + chemotherapy
- Placebo + chemotherapy

Your team can discuss expected benefit, alternatives and uncertainty for your own cancer.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Each phase in your treatment plan

Treatment stages

01 **Paclitaxel + trastuzumab**
First 12 weeks

02 **Trastuzumab alone**
Complete one year in total

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

FIRST 12 WEEKS
Paclitaxel + trastuzumab

First 12 weeks

Weekly chemotherapy visits and three-weekly antibody visits overlap. Premedicines are linked to the paclitaxel dose.

COMPLETE ONE YEAR IN TOTAL
Trastuzumab alone

Complete one year in total

The chemotherapy stops. Heart monitoring continues on the existing schedule. This is adjuvant continuation, not metastatic maintenance.

The active phase determines the medicines and support prescription. Heart monitoring continues through the change to trastuzumab alone.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm that either has happened.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00815 v3 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up · EMA Herceptin product information · formulation and cardiac surveillance

THE WHOLE TREATMENT JOURNEY

Weekly paclitaxel + trastuzumab → trastuzumab continuation: treatment and checks

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment block	During treatment	Change / completion	Follow-up
Cancer medicines NCCP 00815	Confirm exact regimen	Weekly paclitaxel + trastuzumab → trastuzumab continuation	Paclitaxel weekly for 12 doses with trastuzumab every 3 weeks; continue trastuzumab to complete one year of HER2 treatment when appropriate. The team selects IV trastuzumab or locally approved subcutaneous trastuzumab; these are alternative formulations, not additional medicines.	Review active medicines and purpose	Next treatment / follow-up plan
Surgery / radiotherapy NCCP 00815	Adjuvant HER2-positive, node-negative breast cancer; tumour ≤3 cm	Follow the multidisciplinary team (MDT) sequence	Radiotherapy timing needs agreement	Selected local treatment as indicated	Review recovery and ongoing effects
Blood / kidney / liver NCCP 00815	full blood count (FBC), renal and liver profile	Confirm treatment-day results	Repeat on the source/local schedule	Review unresolved abnormalities	Individual clinical plan
Heart / breathing NCCP 00815 · SMPC	Baseline cardiac assessment	Start 12-week monitoring clock	Echo/ multigated acquisition (heart pumping) scan (MUGA) every 12 weeks	Keep clock through phase change	Product-information / risk-based plan

Care / check	Before starting	First treatment block	During treatment	Change / completion	Follow-up
Cancer review NCCP 00815 · EARLY	Confirm stage and setting	Agree review checkpoints	Review symptoms and response	Discuss continuation / next treatment	Breast follow-up
Other checks NCCP 00815 · BONE	Confirm HER2 and baseline eligibility	Review support medicines	Review toxicity before each dose	Endocrine / bone plan where relevant	Follow unresolved symptoms

What each check is for

1

Baseline review

Before treatment

Adjuvant HER2-positive, node-negative breast cancer; tumour ≤3 cm. Assess cardiac function before trastuzumab and every 12 weeks during treatment. Keep the same monitoring clock when paclitaxel ends or trastuzumab changes schedule; arrange earlier review for symptoms or falling function. EMA product information recommends cardiac checks every 6 months after treatment until 24 months; following anthracyclines in early disease, yearly up to 5 years from the last trastuzumab dose or longer if function falls. Your team individualises this plan.

Ask / **act:** Confirm source eligibility and results.

2

Treatment-day check

Before each planned dose

Baseline blood tests and review before each chemotherapy administration according to the source and local plan. Once chemotherapy ends, testing follows trastuzumab and individual clinical needs.

Ask / **act:** Confirm active medicines and whether a dose is held.

3

Support plan

At every phase or day change

Paclitaxel weekly for 12 doses with trastuzumab every 3 weeks; continue trastuzumab to complete one year of HER2 treatment when appropriate. The team selects IV trastuzumab or locally approved subcutaneous trastuzumab; these are alternative formulations, not additional medicines.

Ask / **act:** Choose the matching support script; do not merge phases.

4

Response and follow-up

Agreed review points

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine computed tomography (CT)/positron emission tomography (PET) scans or blood tumour markers are not used to look for distant recurrence in a person without symptoms or clinical concerns.

Ask / **act:** Review symptoms, results and patient priorities together.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Before the first visit

Blood tests, medicines review and the consent discussion come first. Confirm baseline and planned ongoing cardiac assessment.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Paclitaxel is an IV infusion. Trastuzumab may be IV or, if locally approved and prescribed, a subcutaneous injection. Staff confirm formulation, observation and access needs at every phase change.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

Phase-specific visit pattern

Paclitaxel weekly for 12 doses with trastuzumab every 3 weeks; continue trastuzumab to complete one year of HER2 treatment when appropriate. The team selects IV trastuzumab or locally approved subcutaneous trastuzumab; these are alternative formulations, not additional medicines. No take-home infusion pump is part of this schedule. Port de-accessing after a visit is different from removal of the implanted port.

Checks across treatment

Your clinical team books tests, reviews results and explains any change. Contact them if an expected test or instruction is missing.

Blood and kidney tests

Baseline blood tests and review before each chemotherapy administration according to the source and local plan. Once chemotherapy ends, testing follows trastuzumab and individual clinical needs.

Heart and breathing

Assess cardiac function before trastuzumab and every 12 weeks during treatment. Keep the same monitoring clock when paclitaxel ends or trastuzumab changes schedule; arrange earlier review for symptoms or falling function. EMA product information recommends cardiac checks every 6 months after treatment until 24 months; following anthracyclines in early disease, yearly up to 5 years from the last trastuzumab dose or longer if function falls. Your team individualises this plan.

Cancer imaging

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine CT/PET scans or blood tumour markers are not used to look for distant recurrence in a person without symptoms or clinical concerns.

Bone health

Bone-density testing follows endocrine treatment, ovarian suppression, menopause and other bone-risk factors. dual-energy X-ray absorptiometry (bone density) (DEXA) is not automatically required before every chemotherapy course.

The wider treatment plan

Follow the agreed early-breast pathway. Trastuzumab may continue after chemotherapy and can overlap radiotherapy when approved by the treating team. Endocrine therapy depends on hormone receptors.

When treatment changes

Reconcile active cancer medicines, supportive medicines and remaining symptoms before every transition.

Follow-up

Keep the agreed review appointments and report new symptoms between visits; this map does not detect missed tests.

Hormone treatment and bone care

Hormone-receptor results and your treatment setting determine whether endocrine therapy is appropriate. It has its own monitoring plan.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

First 12 weeks · combination

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Four 21-day blocks contain the 12 weekly paclitaxel doses.

Treatment or check	Timing in this view
Paclitaxel Day 1	 **Day 1** IV
Paclitaxel Day 8	 **Day 8** IV
Paclitaxel Day 15	 **Day 15** IV
Trastuzumab	 **Day 1** IV / subcutaneous (SC)
Fatigue / numbness	 **Days 1–21** May accumulate

Treatment bands: Show the planned medicine schedule, subject to review and dose changes.

Checks and restart: Keep the arranged blood / heart appointments. A calendar day alone does not authorise the next dose.

Symptoms: Can occur at any time; broad bands are not measured risk curves.

Why these bands stay broad: Planned treatment days are distinct from broad possible symptom patterns. Symptoms can occur outside these bands. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00815 v3 · official regimen ↗

There are no guaranteed “safe days”. Fever, shivers, sudden illness, new breathing problems or severe symptoms need prompt advice on any day, including a treatment break.

After paclitaxel · trastuzumab alone

Day 1 → Day 21: a general pattern

Trastuzumab continues every 3 weeks to complete one year total.

Treatment or check	Timing in this view
Trastuzumab	 **Day 1** IV / SC
Cardiac symptoms	 **Days 1–21** Report promptly; keep echo checks

Treatment bands: Show the planned medicine schedule, subject to review and dose changes.

Checks and restart: Keep the arranged blood / heart appointments. A calendar day alone does not authorise the next dose.

Symptoms: Can occur at any time; broad bands are not measured risk curves.

> **Why these bands stay broad:** Planned treatment days are distinct from broad possible symptom patterns. Symptoms can occur outside these bands. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00815 v3 · official regimen ↗

Between-treatment notes

Day 1–21: Your reference cycle

Paclitaxel weekly for 12 doses with trastuzumab every 3 weeks; continue trastuzumab to complete one year of HER2 treatment when appropriate. Check the agreed prescription and contact the team for new symptoms. Restart only after the team confirms the plan.

Support around the medicines

1 **Anti-sickness plan**

Follow the prevention and rescue instructions prescribed for this schedule. Call if vomiting prevents drinking.

2 **Infection and bleeding**

Know the local urgent-contact number. Fever, shivering, sudden illness or unusual bleeding needs prompt advice even between visits.

3 **Medicine review**

Show the oncology pharmacist all prescribed, over-the-counter and herbal medicines. Ask before starting anything new.

4 **Premedicines belong to the active phase**

Paclitaxel reaction medicines and AC anti-sickness medicines have different purposes. They are not automatically continued during trastuzumab-only treatment.

Questions worth asking

Who arranges the next tests?

Your treating service arranges and reviews tests. If instructions or appointments are missing, contact the team; the map does not book tests.

What if a dose is missed or I feel unwell?

Contact the unit for a revised plan. Do not double tablets, shift treatment dates or assume that an omitted dose should be made up.

Try explaining the plan back: Which medicines are due on which days? What symptoms mean you should call today? Which tests come next and who arranges them?

Paclitaxel

- Low blood counts and infection
- Numbness / tingling, sometimes lasting
- Hair loss, muscle or joint pain
- Infusion reaction
- Mouth, bowel, skin and nail effects

Trastuzumab

- Heart pumping-function reduction / heart failure
- Infusion or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

First 12 weeks · paclitaxel + trastuzumab

Active medicines: Paclitaxel + Trastuzumab

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).
- From Day 8 onward: chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before treatment. Routine dexamethasone may be omitted after the first dose; prior reaction or increased risk requires consultant review.
- Famotidine may be omitted under the consultant/local plan if appropriate. A prior reaction requires a separate rechallenge assessment; do not reuse the standard draft automatically.
- Trastuzumab formulation and observation instructions remain separate from taxane premedication.
- Growth-factor support during weekly paclitaxel is individualised; do not automatically carry the mandatory dose-dense AC prescription into this phase.
- Confirm IV versus locally approved subcutaneous (SC) trastuzumab. SC is fixed 600 mg every 21 days without a loading dose; IV and SC missed-dose instructions are different.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Adjuvant continuation · trastuzumab alone

Active medicines: Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine chemotherapy antiemetics or paclitaxel premedicines carry into trastuzumab-only continuation. Review previous symptoms/reactions individually.
- Continue the cardiac-monitoring clock and confirm the prescribed route, interval and missed-dose rules.
- Confirm intravenous (IV) versus locally approved subcutaneous (SC) trastuzumab. SC is fixed 600 mg every 21 days without a loading dose; IV and SC missed-dose instructions are different.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Weekly paclitaxel + trastuzumab → trastuzumab continuation

Paclitaxel weekly for 12 doses with trastuzumab every 3 weeks; continue trastuzumab to complete one year of HER2 treatment when appropriate. The team selects intravenous (IV) trastuzumab or locally approved subcutaneous trastuzumab; these are alternative formulations, not additional medicines.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hair / scalp

Chemotherapy

Hair loss or thinning varies; ask about support.

2 Mouth

Weekly paclitaxel + trastuzumab → trastuzumab continuation

Painful ulcers or difficulty drinking need advice.

3 Breathing

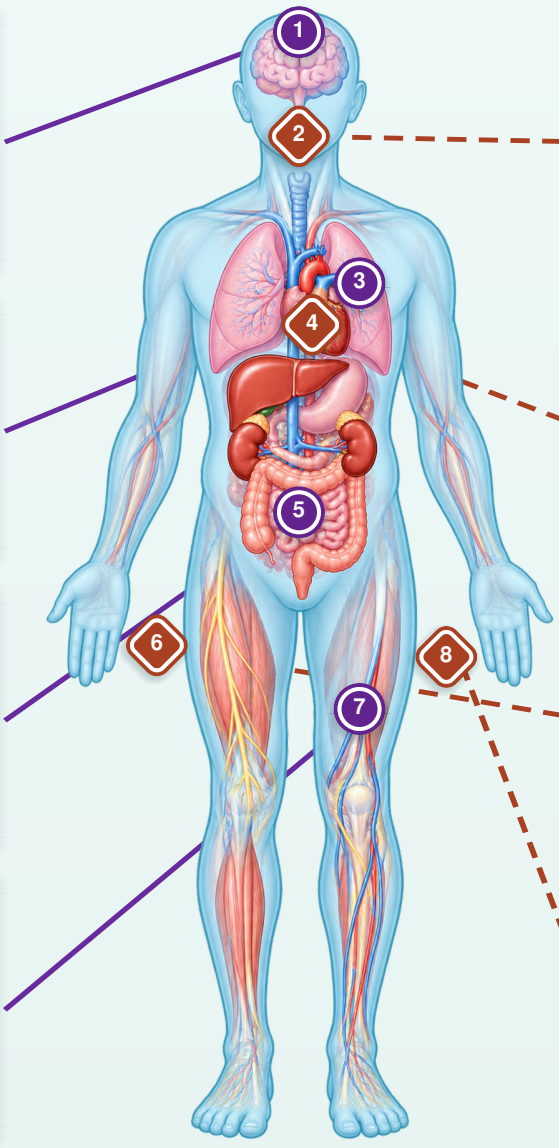
Weekly paclitaxel + trastuzumab → trastuzumab continuation

New cough or breathlessness needs prompt assessment.

4 Heart

Weekly paclitaxel + trastuzumab → trastuzumab continuation

New chest pain, fainting or severe breathlessness needs urgent assessment.



5 Bowel

Weekly paclitaxel + trastuzumab → trastuzumab continuation

Report diarrhoea early, especially with poor intake or dizziness.

6 Nerves / joints

Weekly paclitaxel + trastuzumab → trastuzumab continuation

Report numbness, tingling or pain affecting daily activity.

7 Blood / infection

Weekly paclitaxel + trastuzumab → trastuzumab continuation

Fever, shivers, unusual bleeding or sudden illness: call promptly.

8 Skin / liver symptoms

Weekly paclitaxel + trastuzumab → trastuzumab continuation

Report severe rash, blistering or yellow skin / eyes promptly.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Medicine-specific effects are highlighted; this is not a complete list. New or persistent symptoms still need review after treatment stops.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00815 v3 · official regimen**
   https://healthservice.hse.ie/documents/6427/815_v3_Paclitaxel_weekly_Trastuzumab_3weekly.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
4. **EMA Herceptin product information · formulation and cardiac surveillance**
   https://www.ema.europa.eu/en/documents/product-information/herceptin-epar-product-information_en.pdf
5. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
6. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
7. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
8. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
9. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
10. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf